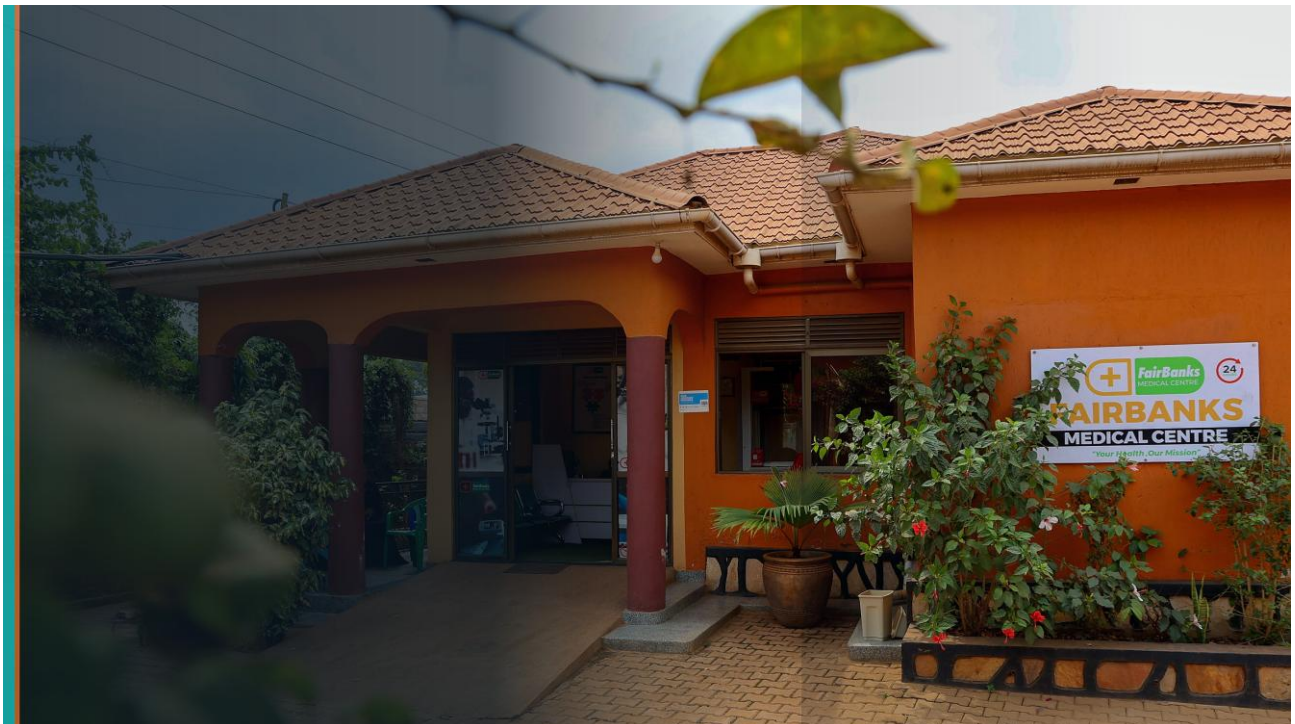




Nexa Climate and Health Initiative 2026

FCHIP Climate Health Early Action

Turning rainfall, heat, and community health signals into timely care in
Uganda



Your health, our mission.

Proof-of-Concept application workbook | Uganda | Working draft

Submission readiness gate

The strongest fit is Proof of Concept. Do not apply as Transition to Scale without credible, quantitative climate-health results. Do not submit until every orange field has been verified by the applicant.

Critical item	Current position
Applicant legal name and entity type	[CONFIRM BEFORE SUBMISSION]
Incorporated, active, and in good standing in Uganda	[CONFIRM BEFORE SUBMISSION]
Project Lead identity, role, email, and one-application confirmation	[CONFIRM BEFORE SUBMISSION]
Requested amount and 18/21/24-month duration	[CONFIRM BEFORE SUBMISSION]
Collaborators and written roles	[CONFIRM BEFORE SUBMISSION]
Climate/weather data source and permission or licence	[CONFIRM BEFORE SUBMISSION]
District, facility, community, ethics, and data approvals	[CONFIRM BEFORE SUBMISSION]
Baseline values, sample, targets, and budget	[CONFIRM BEFORE SUBMISSION]
Prior GCC applications/awards and third-party IP	[CONFIRM BEFORE SUBMISSION]

1. Official call brief

Item	Verified information
Call	Nexa - Climate and Health Innovation: Africa, Latin America and the Caribbean
Co-leads	Grand Challenges Canada and the Science for Africa Foundation
Deadline	22 July 2026, 2:00 p.m. ET / 6:00 p.m. UTC
Submission	Fluxx only; complete application in English or French
Recommended stage	Proof of Concept; current evidence does not support Transition to Scale
Amount	Up to USD 200,000
Duration	18, 21, or 24 months
Applicant	Incorporated or equivalent in Africa or Brazil; Uganda is eligible
Implementation	Uganda is an eligible Proof-of-Concept implementation country
Community threshold	At least community linked

1.1 Why FCHIP can fit

- Direct fit with climate-informed early warning and monitoring systems.
- A clear path from climate and health signals to named service-delivery actions.

- Existing medical centre, outreach activity, and CHW/VHT links support field testing.
- Uganda is eligible and the target groups match Nexa's climate-vulnerable priorities.
- The proposal must prove climate integration, not simply add weather data to a health app.

1.2 Relevant hazards and outcomes

Nexa scope	Proposed focus
Changing mosquito ecology	Malaria risk, prevention, testing, referral, and supply readiness
Extreme heat	Pregnancy hypertension/heat stress, cardiovascular disease, and diabetes care
Poor air quality	Out of initial scope unless reliable local data and respiratory workflow are confirmed
Priority groups	Pregnant women, children, older people, underserved urban communities, climate-sensitive NCDs

2. Proposed investment details

Portal field	Draft / action
Applicant organisation	[CONFIRM BEFORE SUBMISSION]
Project Lead	[CONFIRM BEFORE SUBMISSION] - formally affiliated, key team member, one application only
Project title (100 characters)	FCHIP Climate Health Early Action
Amount requested	[CONFIRM BEFORE SUBMISSION] - maximum USD 200,000
Duration	[CONFIRM BEFORE SUBMISSION] - select 18, 21, or 24 months
Collaborators	[CONFIRM BEFORE SUBMISSION] - list role and legal name
Country of incorporation	Uganda - confirm against incorporation documents
Legal entity type	[CONFIRM BEFORE SUBMISSION]
Country of implementation	Uganda
Community connection	[CONFIRM BEFORE SUBMISSION] - likely community partnered/linked; choose from evidence
Area of focus	Climate-informed early warning and monitoring systems
Language	English

3. Innovation Overview - exact character-limited answer bank

Only paste verified text. Character counts below include spaces and exclude the question label. Keep a small buffer because Fluxx may count formatting differently.

1. Specific climate-driven health gap [1,500]

Climate change is making malaria risk less predictable in Kampala and surrounding communities. Shifting rainfall, temperature, and humidity can change mosquito breeding and transmission, while extreme heat adds risk for

pregnant women and people living with cardiovascular disease or diabetes. Yet local health actors mainly see separate, late signals: CHW and VHT reports, outreach screening records, facility visits, medicine use, and weather information. These signals are rarely combined into simple triggers that tell a CHW, clinic, or programme when and where to act. Underserved households, including pregnant women, children, older people, and residents of informal urban communities, therefore face delayed prevention, testing, referral, follow-up, and stock preparation. FairBanks needs a practical local system that turns climate-driven risk signals into timely health service action, not another dashboard that stops at an alert.

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2. Proposed innovation [2,000]

FCHIP Climate Health Early Action is a climate-integrated extension of the FairBanks Community Health Intelligence Platform. It will combine approved rainfall, temperature, humidity, and seasonal data with structured CHW/VHT reports, outreach screening, facility trends, and medicine-use signals. An offline-capable mobile workflow will help frontline workers collect and review essential data. Explainable risk rules will first identify time- and place malaria risk and heat-sensitive risk for pregnancy, cardiovascular disease, and diabetes. A GIS action board will show risk, data quality, and agreed response triggers. Each trigger will connect to a protocol: targeted health messages, household follow-up, malaria testing/referral, outreach scheduling, clinician review, or medicine and supply preparation. The Proof of Concept will test whether this end-to-end loop is feasible, accepted, timely, and capable of improving access to climate-responsive care. Models will not make diagnoses or replace clinical judgement.

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3. Population design and access [1,500]

The pilot will focus on climate-vulnerable households in FairBanks Community Reach areas in and around Kampala, with priority attention to pregnant women, children, older people, and people living with cardiovascular disease or diabetes. These groups may be more exposed to mosquito-borne infection, heat stress, disrupted care, and financial barriers. CHWs and VHTs will help identify needs, test language and workflows, explain consent, collect only necessary data, and connect households to services. Community members will join design sessions, usability tests, feedback meetings, and review of what actions followed alerts. Access will not depend on owning a smartphone: frontline workers will use offline-capable tools and existing outreach channels. Participation targets, locations, safeguarding steps, disability access, and gender-responsive engagement must be confirmed with communities before submission and implementation.

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4. Innovation in the target setting [2,000]

Current approaches often keep weather information, community reports, and clinical records in separate systems. Some early warning tools stop at forecasting and leave local teams to decide what to do. FCHIP is designed around the full early-action chain: community signal, climate signal, explainable trigger, named actor, service response, and follow-up result. It combines technological innovation (offline capture, secure data integration, interpretable analytics, and GIS), social innovation (CHW/VHT-led design and community feedback), and a delivery model rooted in a functioning medical centre and Community Reach programmes. It is a better fit for a lower-resource setting because it starts with simple validated rules, works with intermittent connectivity, and keeps clinicians responsible for care decisions. The pilot will compare timeliness, completion, acceptability, and cost with current manual planning before adding any machine learning.

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5. Potential to improve priority outcomes [2,000]

The primary aim is improved and more timely access to prevention, testing, referral, monitoring, and treatment for climate-sensitive conditions. For malaria, rainfall and community symptom signals should trigger earlier targeted communication, testing, outreach, and supply preparation. For heat-sensitive pregnancy, cardiovascular disease, and diabetes, heat thresholds should trigger follow-up, hydration and risk messages, clinician review, and

continuity-of-care checks. The pilot will test whether more at-risk people receive a documented service action after a climate-informed alert, whether response time falls, and whether referral and follow-up completion improve. It will also track false alerts, missed events, equity of reach, service continuity, and unintended harm. Final baseline values and realistic targets must be set from verified FairBanks and partner data; technical alert accuracy alone will not count as health impact.

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6. Climate influence on design and deployment [1,500]

FCHIP began as a community health intelligence concept and is being fundamentally re-engineered for this pilot as a climate-integrated health innovation. Climate factors will shape its design through rainfall, temperature, humidity, seasonality, and agreed hazard thresholds; its functioning through climate-sensitive risk rules and alert timing; and its deployment through risk-based outreach, staffing, follow-up, and supply preparation. The innovation will be tested across changing climate-risk periods rather than as a static health register. The team will document whether performance changes during heavy rainfall, heat, poor connectivity, or service pressure. The exact climate data provider, geographic resolution, historical coverage, update frequency, licences, and locally approved thresholds must be confirmed before submission.

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7. Team and community connection [1,500]

FairBanks operates a medical centre and Community Reach activities serving Kampala-area communities including Bukoto, Kyebando, Kisaasi, Kamwokya, and Kikaaya. Its model links community members, CHWs and VHTs, outreach programmes, clinical care, research, and skills development. This gives the project a local service presence and a practical route for community design, referral, and follow-up. The application must still name the Project Lead, technical and clinical leads, CHW/VHT representatives, MEL support, data protection responsibility, and confirmed collaborators. It must describe each person's training, lived experience, location, role, and prior partnership outcomes. Do not claim community-owned or community-led status until leadership and residence evidence confirms it.

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4. Full Proof-of-Concept application

These fields are reviewed after the Innovation Screen. They are application-ready structures, but facts and numbers marked for confirmation must be supplied before submission.

1. Execution plan [3,000]

Months 1-3: confirm governance, communities, users, climate data, baseline, protocols, ethics and safeguarding; co-design workflows. Months 4-7: build and test offline forms, data pipeline, explainable rules, GIS action board, and audit trail. Months 8-10: train users and run a small usability and data-quality test. Months 11-18: pilot agreed malaria and heat-sensitive care workflows across selected sites and seasons, with monthly safety and learning reviews. Months 19-21: analyse quantitative and qualitative results, refine thresholds and workflows, and assess cost and sustainability. Months 22-24: validate findings with communities and partners, publish a responsible evidence package, and make a scale/no-scale decision. Exact sites, sample, milestones, owners, and duration: [CONFIRM BEFORE SUBMISSION]

2. Community engagement [2,000]

Use paid or otherwise fairly supported CHW/VHT and community representation in design, testing, interpretation, and governance. Hold separate listening sessions where needed for pregnant women, older people, people with disabilities, and low-income residents. Test language, consent, alert burden, referral barriers, and preferred communication. Publish a feedback route and show what changed. Track participation by sex, age, disability where appropriate, and location without collecting unnecessary personal data. Community representatives, compensation, safeguarding route, and engagement targets: [CONFIRM BEFORE SUBMISSION]

3. Risks and mitigation [2,000]

Main risks are weak or biased data, false or missed alerts, alert fatigue, privacy harm, poor connectivity, low adoption, exclusion, unsafe referrals, partner delay, climate-data licensing, corruption, and supply constraints after an alert. Mitigation includes data minimisation, consent, role access, encryption, audit logs, clinician oversight, threshold validation, offline workflows, escalation protocols, safeguarding, complaints handling, anti-fraud controls, partner agreements, and pause criteria. Applicable Uganda approvals, ethics review, incident response, insurance, and organisational policies: [CONFIRM BEFORE SUBMISSION]

4. Expected impact [2,000]

Expected results are improved capacity to interpret climate-health signals; faster action after risk triggers; more at-risk people reached with prevention, testing, referral, or continuity-of-care support; improved referral completion; and better preparation of outreach and supplies. The pilot will report reach and outcomes by vulnerable group and location. It will not promise reductions in illness or hospitalisation without a powered evaluation. Verified baseline, sample size, effect assumptions, targets, and attribution approach: [CONFIRM BEFORE SUBMISSION]

5. Proof of Concept and objectives [2,000]

Establish that an end-to-end climate-informed workflow is technically feasible, acceptable, safe, affordable enough to continue, and able to improve timely access to service actions. Proposed objectives: (1) achieve an agreed completeness and timeliness level for core data; (2) validate malaria and heat-risk triggers against observed data; (3) reduce median time from trigger to documented action; (4) improve referral/follow-up completion; (5) reach priority groups equitably without serious safety or privacy events; and (6) secure a costed continuation decision from FairBanks and confirmed partners. Numeric targets: [CONFIRM BEFORE SUBMISSION]

6. Monitoring and evaluation [2,500]

Use a prospective mixed-methods pilot with a pre-defined theory of change. Compare baseline and implementation periods and, if feasible, matched workflows or sites. Measure data quality, alert performance, response time, service actions, referrals, continuity of care, reach, equity, acceptability, adoption, cost per person reached, and safety events. Sources include system logs, CHW forms, facility records, referral registers, climate data, surveys, interviews, and focus groups. Use confidence intervals and transparent missing-data analysis; report false positives and negatives. An independent statistician or research partner, final design, power/sample calculation, tools, approvals, and baseline: [CONFIRM BEFORE SUBMISSION]

7. Sustainability [2,000]

Build on existing FairBanks outreach and clinical workflows, use low-cost offline tools, train local users, document protocols, and test willingness and ability to pay. Future routes may include clinic subscriptions, programme implementation contracts, district or NGO deployments, and approved integrations. Sustainability depends on evidence, data cost, maintenance, workforce time, public-system fit, and reliable service capacity after alerts. The pilot will produce total-cost-of-ownership and financing scenarios, not assume grant-free scale. Confirmed buyers, government pathway, partner commitments, and pricing evidence: [CONFIRM BEFORE SUBMISSION]

8. Project team [1,500]

FairBanks contributes local primary-care delivery, outreach, CHW/VHT links, and community follow-up. The project requires named leadership across clinical care, climate/epidemiology, product engineering, data protection, MEL, community engagement, finance, safeguarding, and partnerships. Attach roles and time commitments and close gaps through collaborators. Names, qualifications, employment links, CV evidence, and collaborator commitments: [CONFIRM BEFORE SUBMISSION]

9. Prior partnerships [1,500]

Describe only verified work with community groups, health facilities, government, academic, private-sector, and development partners. State each partner, dates, role, activity, and result. Repository material supports a community-health ecosystem but does not verify every formal partnership. Evidence and approved wording: [CONFIRM BEFORE SUBMISSION]

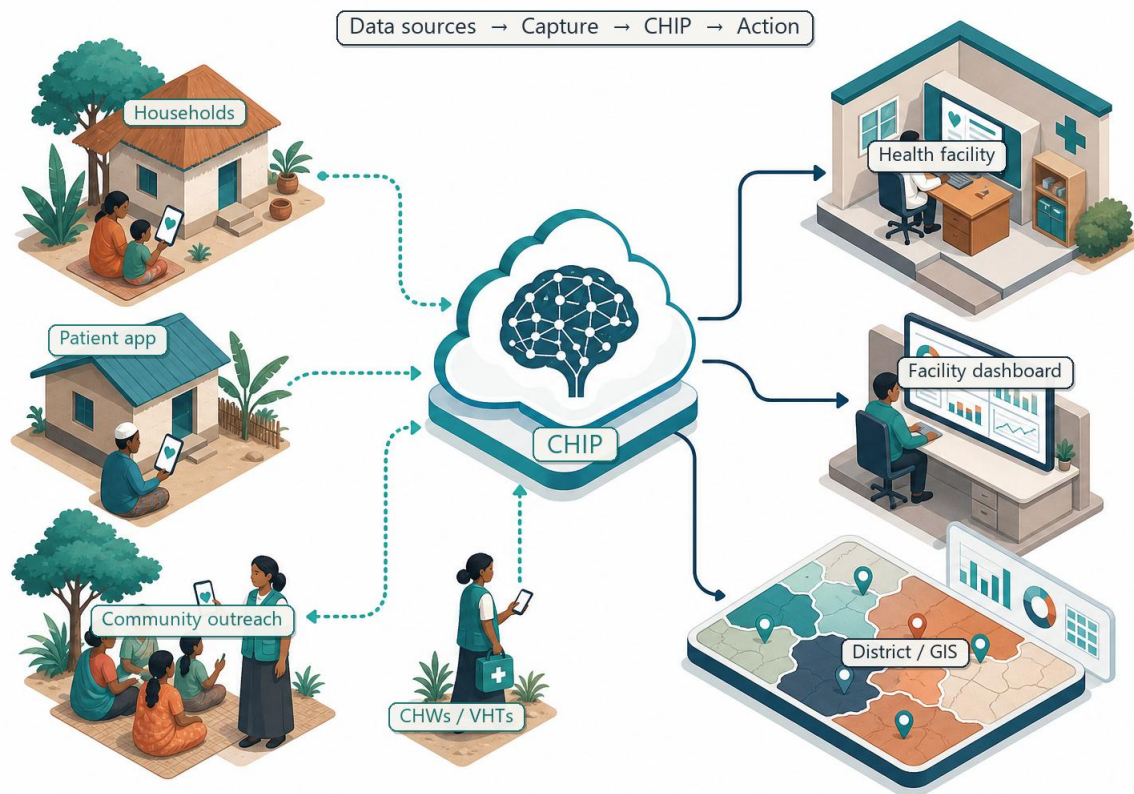
10. Budget

Complete Fluxx categories: remuneration; subcontractor fees; travel; goods and supplies; equipment; project administration; sub-grants; and indirect costs capped at 10% of direct costs. Certify that the budget is complete and that most activities and expenses occur in the eligible implementation country. Detailed amount, currency assumptions, quotations, indirect-cost basis, and majority-spend confirmation: [CONFIRM BEFORE SUBMISSION]

4.1 Budget workbook

Fluxx category	Amount (USD)	Description / basis
Remuneration	[CONFIRM BEFORE SUBMISSION]	[CONFIRM BEFORE SUBMISSION]
Subcontractor fees	[CONFIRM BEFORE SUBMISSION]	[CONFIRM BEFORE SUBMISSION]
Travel costs	[CONFIRM BEFORE SUBMISSION]	[CONFIRM BEFORE SUBMISSION]
Goods and supplies	[CONFIRM BEFORE SUBMISSION]	[CONFIRM BEFORE SUBMISSION]
Equipment costs	[CONFIRM BEFORE SUBMISSION]	[CONFIRM BEFORE SUBMISSION]
Project administration costs	[CONFIRM BEFORE SUBMISSION]	[CONFIRM BEFORE SUBMISSION]
Sub-grants	[CONFIRM BEFORE SUBMISSION]	[CONFIRM BEFORE SUBMISSION]
Indirect costs	[CONFIRM BEFORE SUBMISSION]	Maximum 10% of direct costs - verify calculation
Total	[CONFIRM BEFORE SUBMISSION]	Must not exceed USD 200,000

5. Product and action pathway



Illustrative FCHIP architecture; not evidence of a deployed climate-health product.

Layer	Proof-of-Concept role
Climate inputs	Rainfall, temperature, humidity, seasonality, and approved thresholds
Health inputs	CHW/VHT reports, outreach screening, facility trends, and medicine use
Responsible analytics	Explainable rules first; validation, monitoring, and clinician review
Action layer	GIS risk board, named response owner, protocol, referral, and follow-up
Learning loop	Response outcome, data quality, equity, safety, cost, and threshold refinement

5.1 Responsible AI and clinical safety

- Start with explainable thresholds and rules; add machine learning only after sufficient data and validation.
- Do not diagnose, prescribe, or automatically deny or delay care.
- Show uncertainty, data quality, and the reason for each alert.
- Require clinical review for care decisions and define escalation and pause criteria.
- Monitor bias, false alerts, missed events, workload, privacy, and unintended harm.

6. Monitoring, evaluation, and learning



Illustrative dashboard concept; measures and thresholds remain to be validated.

Domain	Measures
Data readiness	Completeness, timeliness, missingness, geographic coverage
Alert performance	Sensitivity, specificity, false alerts, missed events, lead time
Health-system response	Time to action, actions completed, referral and follow-up completion
Priority outcomes	Timely prevention, testing, care access, continuity, complications where feasible
Equity and safety	Reach by priority group, complaints, adverse events, privacy incidents
Feasibility	Adoption, acceptability, uptime, cost per person reached, staff burden

Nexa requires meaningful health access or health outcome evidence. Forecast accuracy alone is not enough.

7. Community-led delivery



Community needs guide CHW/VHT workflows, service action, and learning.

- Community members identify needs and test the usability, language, and fairness of workflows.
- CHWs and VHTs bridge households to prevention, screening, referral, and follow-up.
- Community Reach programmes coordinate outreach and health education.
- FairBanks Medical Centre provides clinical review, diagnostics, pharmacy, referral, and quality assurance.
- Research and partners support evidence, climate expertise, training, and ethical evaluation.
- Learning returns to communities and strengthens resilience, not only project reporting.

8. Risks and safeguards

Risk	Mitigation
Overclaiming climate fit	Make climate data and hazard-responsive action central, not decorative.
Weak evidence	Set baseline, comparison logic, sample, tools, and analysis before launch.
Unsafe automation	No diagnosis; clinician oversight, conservative thresholds, and pause rules.
Privacy and ethics	Minimise data, obtain consent/approval, control access, and log activity.
Exclusion	Offline CHW-mediated access; disability, gender, language, and poverty checks.
No capacity after alert	Pre-agree service protocols, owners, supplies, referral routes, and escalation.
Financial misuse	Segregated duties, procurement controls, declarations, audit trail, and reporting.

9. Additional portal information

Field	Action
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Previous GCC application	[CONFIRM BEFORE SUBMISSION] - Yes/No and details
Previous GCC award	[CONFIRM BEFORE SUBMISSION] - Yes/No, grant IDs, and difference from prior work
Citations	Add recent, relevant climate-malaria, heat-health, CHW, and early-action evidence
Third-party IP	[CONFIRM BEFORE SUBMISSION] - include software, models, maps, climate and health datasets
Licences/access	[CONFIRM BEFORE SUBMISSION] - verify rights allow implementation and future scale
Certification	Authorised signatory must accept call terms and privacy notice
Confidentiality	Do not submit trade secrets or sensitive data that are unnecessary for review
AI disclosure	Applicant remains responsible for original, true, accurate, complete, and cited content

10. Final submission checklist

- Recheck the live call, deadline, and any revised documents.
- Register in Fluxx early and confirm the legal organisation record.
- Confirm Proof-of-Concept stage, amount, duration, and Uganda implementation.
- Replace every CONFIRM BEFORE SUBMISSION marker.
- Keep all seven Innovation Overview answers within character limits.
- Verify that climate factors shape design, function, and deployment.
- Link every risk signal to a named health service action and outcome.
- Complete the 24-month plan, MEL design, objectives, safeguards, and budget.
- Verify Project Lead, team, collaborators, community role, approvals, IP, and prior GCC history.
- Review for confidential information, plagiarism, unsupported claims, and citation quality.
- Submit in Fluxx before the deadline; save the confirmation.

11. Official sources

Source	URL
Official Nexa call page	https://www.grandchallenges.ca/rfp-nexa/
English Funding Opportunity	https://www.grandchallenges.ca/wp-content/uploads/2026/06/EN-Nexa-RFP-Jun-18-2026.pdf
Proof-of-Concept questions	https://www.grandchallenges.ca/wp-content/uploads/2026/06/Nexa-POC-application_for-webpage.pdf
Transition-to-Scale questions	https://www.grandchallenges.ca/wp-content/uploads/2026/06/Nexa-TTS-Stage-1-application_for-webpage.pdf
Fluxx application portal	https://gcc.fluxx.io

Source check date: 2026-07-19. Official Nexa documents and Fluxx always win.